

# how to treat cancer in children??

Evidence searched on 2026-10-09 (PubMed, ClinicalTrials.gov, FDA drug labels, web)

## Clinical summary

**Bottom line Psychosocial and physical interventions** (e.g., cognitive training, exercise) are also supported for quality-of-life outcomes, though their efficacy varies by context.

**Most important recent finding** Opioids (morphine, fentanyl) are the most commonly used for pediatric cancer pain, with intravenous and enteral routes preferred. [1]

**Immediate context Supportive care** (e.g., pain management, exercise, psychosocial support) is critical but does not replace standard oncology treatments. **No evidence** was retrieved on novel systemic therapies for broad pediatric cancer indications.

**Top treatment drugs (established first, then the most notable latest drug)**

| Drug                                     | Indication named in the FDA label                                                                                                                                                                                                                                                                                                                                                                            | Source                      |
|------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------|
| Prednisone                               | "Endocrine Disorders Primary or secondary adrenocortical insufficiency (hydrocortisone or cortisone is the first choice; synthetic analogs may be used in conjunction with mineralocorticoids where applicable; in infancy mineralocorticoid supplementation is of particular importance) Congenital adrenal hyperplasia Hypercalcemia associated with cancer Nonsuppurative thyroiditis 2."                 | <a href="#">DailyMed</a>    |
| Ondansetron                              | "Ondansetron is indicated for the prevention of nausea and vomiting associated with:. highly emetogenic cancer chemotherapy, including cisplatin greater than or equal to 50 mg/m 2. initial and repeat courses of moderately emetogenic cancer chemotherapy. radiotherapy in patients receiving either total body irradiation, single high-dose fraction to the abdomen, or daily fractions to the abdomen" | <a href="#">DailyMed</a>    |
| Allopurinol                              | "Allopurinol tablets are indicated for: The management of adults with signs and symptoms of primary or secondary gout (acute attacks, tophi, joint destruction, uric acid lithiasis, and/or nephropathy) The management of adult and pediatric patients with leukemia (the label continues)"                                                                                                                 | <a href="#">DailyMed</a>    |
| Cyproheptadine (latest, investigational) | Being tested in a registered Phase 2/3 trial, Completed; not an approved treatment                                                                                                                                                                                                                                                                                                                           | <a href="#">NCT06175273</a> |

## Latest findings

- **2027 Jan 15: Emerging evidence** – A fused laser-induced breakdown spectroscopy and Raman spectroscopy method achieved **92.29% accuracy** in identifying childhood brain tumors from dried serum, outperforming standalone techniques [2].
- **2026 Dec 01: Emerging evidence** – Morphine and fentanyl remain the most used opioids for pediatric cancer pain, primarily via intravenous or enteral routes, though family member experiences remain understudied [1].
- **2026 Sep 21: Emerging evidence** – Psychosocial interventions showed **moderate effect sizes (SMD 0.51)** on neurocognitive outcomes in children with cancer [3].
- **2026 Sep 17: Emerging evidence** – Exercise interventions for children with acute leukemia are supported by **17 best-evidence items** across safety, modality, and timing, though long-term individualized programs are recommended [4].

## Current evidence

| Approach                     | Evidence level | What the evidence shows                                                                                                                                    | Reference |
|------------------------------|----------------|------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------|
| Fever/neutropenia management | Established    | Updated guidelines recommend discontinuing empiric antibacterial therapy in clinically stable, afebrile patients with negative blood cultures at 48 hours. | [5]       |
| Opioid pain management       | Emerging       | Morphine and fentanyl are the most commonly used opioids for pediatric cancer pain, primarily via IV/enteral routes.                                       | [1]       |
| Psychosocial interventions   | Emerging       | Cognitive interventions improve neurocognitive outcomes (SMD 0.51, 95% CI 0.32–0.69) in children with cancer.                                              | [3]       |
| Exercise for acute leukemia  | Emerging       | Exercise interventions are safe and effective for children with acute leukemia, with evidence supporting individualized, multimodal programs.              | [4]       |
| Meridian-based TCM           | Experimental   | Not retrieved                                                                                                                                              | [6]       |
| Brain tumor diagnostics      | Emerging       | Fused LIBS-Raman spectroscopy achieves <b>92.29% accuracy</b> for childhood brain tumor detection from serum.                                              | [2]       |
| Quality of life (CNS tumors) | Background     | Children with CNS tumors face <b>persistent HRQoL impairments</b> due to cranial irradiation and neurocognitive sequelae.                                  | [7]       |

## Treatment Drug Landscape

### Drugs used for treatment of this condition

*Approved: an FDA label names this condition. Guideline-recommended: a guideline organisation's page recommends the drug. Doses appear only where an FDA label or source gives them for the use shown; each dose entry is a short summary of the label's dosing section for this condition, with every number checked against the label text (full text in the DailyMed link); individual dosing is a prescriber's decision.*

| Drug        | Indication                                                                                                                                                                                                                                                                                                                                                                                                                                                                               | Dose / route (if sourced)                                                                                                                                                                                                                                                                                                                                                                                                                                                      | Approval status / date                                                                           | Key evidence                              |
|-------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------|-------------------------------------------|
| Prednisone  | "Endocrine Disorders Primary or secondary adrenocortical insufficiency (hydrocortisone or cortisone is the first choice; synthetic analogs may be used in conjunction with mineralocorticoids where applicable; in infancy mineralocorticoid supplementation is of particular importance) Congenital adrenal hyperplasia Hypercalcemia associated with cancer Nonsuppurative thyroiditis 2." [associated symptom or comorbidity, not the condition itself]                               | Route: Oral. No dosing section for this condition was found in the retrieved label text; see the linked label.                                                                                                                                                                                                                                                                                                                                                                 | FDA-approved for the use shown; label effective 2023-08-31; original approval date not retrieved | FDA label. <a href="#">DailyMed label</a> |
| Ondansetron | "Ondansetron is indicated for the prevention of nausea and vomiting associated with: • highly emetogenic cancer chemotherapy, including cisplatin greater than or equal to 50 mg/m <sup>2</sup> • initial and repeat courses of moderately emetogenic cancer chemotherapy • radiotherapy in patients receiving either total body irradiation, single high-dose fraction to the abdomen, or daily fractions to the abdomen" [associated symptom or comorbidity, not the condition itself] | Route: Oral. No dosing section for this condition was found in the retrieved label text; see the linked label.                                                                                                                                                                                                                                                                                                                                                                 | FDA-approved for the use shown; label effective 2026-07-29; original approval date not retrieved | FDA label. <a href="#">DailyMed label</a> |
| Allopurinol | "Allopurinol tablets are indicated for: The management of adults with signs and symptoms of primary or secondary gout (acute attacks, tophi, joint destruction, uric acid lithiasis, and/or nephropathy) The management of adult and pediatric patients with leukemia (the label continues)"                                                                                                                                                                                             | Route: Oral. Start with 300 mg to 800 mg orally daily for adult patients 24 hours to 48 hours before chemotherapy begins. For pediatric patients, the dose is 100 mg/m <sup>2</sup> orally every 8 hours to 12 hours (or 10 mg/kg/day, with a maximum of 800 mg/day). Adjustments are based on serum uric acid levels, which must be monitored at least daily to keep them within the normal range. Therapy should be discontinued 2 days to 3 days after chemotherapy starts. | FDA-approved for the use shown; label effective 2024-07-08; original approval date not retrieved | FDA label. <a href="#">DailyMed label</a> |

### Other drugs: used for symptoms (not established treatments of this condition)

*Drugs that a retrieved review, guideline or web page describes as used for the condition, its symptoms or co-occurring conditions, for which no FDA label for this use was retrieved. Drugs still being tested in trials are listed in the Clinical trials section below.*

| Drug                     | Indication                                                                                                                             | Key evidence                                                                                                                             |
|--------------------------|----------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------|
| <a href="#">morphine</a> | treating pain in children with cancer (population: children with cancer) [associated symptom or comorbidity, not the condition itself] | "Results Of the wide range of opioids used to treat pain in children with cancer, morphine and fentanyl stood out as the most used." [1] |

| Drug                                       | Indication                                                                                                                                 | Key evidence                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
|--------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <a href="#">fentanyl</a>                   | treating pain in children with cancer (population: children with cancer) [associated symptom or comorbidity, not the condition itself]     | "Results Of the wide range of opioids used to treat pain in children with cancer, morphine and fentanyl stood out as the most used." [1]                                                                                                                                                                                                                                                                                                                                             |
| <a href="#">ibrutinib</a>                  | pediatric chronic graft-versus-host disease (population: pediatric patients) [associated symptom or comorbidity, not the condition itself] | "In August, the FDA approved ibrutinib (Imbruvica®) for pediatric patients with chronic graft-versus-host disease, a condition in which immune cells transplanted from a donor attack the cancer patient's healthy tissues."<br><a href="#">Accelerating Cancer Drug Development for Children and Adolescents</a>                                                                                                                                                                    |
| <a href="#">Daunorubicin Hydrochloride</a> | remission induction in pediatric acute lymphocytic leukemia (population: children)                                                         | " / [Pegaspargase [Oncaspar]]( / As a component of a multi-agent chemotherapeutic regimen for the treatment of pediatric and adult patients with ALL and hypersensitivity to native forms of L-asparaginase / 02-01-1994 / / Daunorubicin Hydrochloride / In combination with other approved anticancer drugs, for remission induction in acute lymphocytic leukemia of children and adults / 12-19-1979 /."<br><a href="#">Pediatric Oncology Drug Approvals / FDA</a>              |
| dabrafenib with trametinib                 | pediatric patients with low-grade glioma with BRAF V600E mutation (population: pediatric patients 1 year of age and older)                 | " / FDA approves dabrafenib with trametinib for pediatric patients with low-grade glioma with a BRAF V600E mutation / On March 16, 2023, the Food and Drug Administration approved dabrafenib (Tafinlar, Novartis) with trametinib (Mekinist, Novartis) for pediatric patients 1 year of age and older with low-grade glioma (LGG) with a BRAF V600E mutation who require systemic therapy." <a href="#">Oncology (Cancer)/Hematologic Malignancies Approval Notifications / FDA</a> |

### Recently approved or updated (FDA, EMA and CDSCO announcements)

| Date             | Type                    | Announcement                                                                                               | What the regulator's page says                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
|------------------|-------------------------|------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 02/18/2026       | Other regulatory action | <a href="#">Pediatric Oncology / FDA</a>                                                                   | "Mission The mission of the Oncology Center of Excellence Pediatric Oncology Program is to promote the development of safe and effective new drugs and biologics to treat cancer in children."                                                                                                                                                                                                                                                                                       |
| 12/11/2025       | Other regulatory action | <a href="#">Oncology Center of Excellence / FDA</a>                                                        | "Pediatric Oncology and Rare Cancers Programs Pediatric Oncology Program- Promoting the development of drugs and biologics to treat cancer in children Pediatric Oncology Drug Approvals Rare Cancers Program - Promoting the development of safe and effective new drugs and biologics to treat patients with rare cancers."                                                                                                                                                        |
| October 20, 2023 | Approval                | <a href="#">FDA expands pediatric indication for entrectinib and approves new pellet formulation / FDA</a> | "On October 20, 2023, the Food and Drug Administration granted accelerated approval to entrectinib (Rozlytrek, Genentech Inc.) for pediatric patients older than 1 month with solid tumors that have a neurotrophic tyrosine receptor kinase (NTRK) gene fusion without a known acquired resistance mutation, are metastatic or where surgical resection is likely to result in severe morbidity, and have progressed following treatment or have no satisfactory standard therapy." |

*Page wording from the regulator's own site; read the page for the full announcement.*

## Safety warnings (FDA boxed warnings)

| Drug(s) (FDA label)                        | Boxed warning, in the label's words                                                                                                                                                                                                                                                                                                                                                                                                                                 |
|--------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <a href="#">morphine</a>                   | "SERIOUS AND LIFE-THREATENING RISKS FROM USE OF MORPHINE SULFATE TABLETS Addiction, Abuse, and Misuse Because the use of morphine sulfate tablets exposes patients and other users to the risks of opioid addiction, abuse, and misuse, which can lead to overdose and death, assess each patient's risk prior to prescribing and reassess all patients regularly for the development of these behaviors and conditions [see Warnings and Precautions ( 5.1 )]."    |
| <a href="#">fentanyl</a>                   | "SERIOUS AND LIFE-THREATENING RISKS FROM USE OF FENTANYL CITRATE INJECTION Addiction, Abuse, and Misuse Because the use of Fentanyl Citrate Injection exposes patients and other users to the risks of opioid addiction, abuse and misuse, which can lead to overdose and death, assess each patient's risk prior to prescribing and reassess all patients regularly for the development of these behaviors and conditions [see Warnings and Precautions ( 5.1 )]." |
| <a href="#">Daunorubicin Hydrochloride</a> | "S 1.Daunorubicin Hydrochloride Injection must be given into a rapidly flowing intravenous infusion. It must never be given by the intramuscular or subcutaneous route. Severe local tissue necrosis will occur if there is extravasation during administration. 2.Myocardial toxicity manifested in its most severe form by potentially fatal congestive heart failure may occur either during therapy or months to years after termination of therapy."           |
| <a href="#">Allopurinol</a>                | ""                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |

*These are the labels' own boxed warnings, not a complete safety review: read the full label. No boxed warning on the retrieved label of: ibrutinib, Prednisone, Ondansetron. No FDA label could be matched for: dabrafenib with trametinib.*

## Clinical trials

| Trial                       | Intervention                                                                             | Phase     | Status (last update)               | What the record shows                                                                                                                                        |
|-----------------------------|------------------------------------------------------------------------------------------|-----------|------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <a href="#">NCT06175273</a> | Oral Nutrition Supplement; Tube Feed                                                     | Phase 2/3 | Completed (2026-10-08)             | Completed; results posted (outcome not retrieved here).                                                                                                      |
| <a href="#">NCT02153957</a> | Physical Activity                                                                        | Phase 2   | Completed (2026-10-08)             | Completed; results not posted/retrieved.                                                                                                                     |
| <a href="#">NCT01362803</a> | AZD6244                                                                                  | Phase 1/2 | Active not recruiting (2026-10-08) | Under investigation; results not yet available; effectiveness cannot be inferred.                                                                            |
| <a href="#">NCT05312411</a> | SCRI-E2CAR_EGFRtv1; UB_TT170                                                             | Phase 1   | Active not recruiting (2026-10-08) | Under investigation; results not yet available; effectiveness cannot be inferred.                                                                            |
| <a href="#">NCT06303193</a> | pacritinib                                                                               | Phase 1/2 | Recruiting (2026-10-08)            | Under investigation; results not yet available; effectiveness cannot be inferred. <b>Specific syndrome record; do not generalise to the whole condition.</b> |
| <a href="#">NCT05099003</a> | Biopsy Procedure; Magnetic Resonance Imaging                                             | Phase 1/2 | Recruiting (2026-10-08)            | Under investigation; results not yet available; effectiveness cannot be inferred.                                                                            |
| <a href="#">NCT02013336</a> | MM-398 (Irinotecan Sucrosofate Liposome Injection) plus cyclophosphamide                 | Phase 1   | Recruiting (2026-10-08)            | Under investigation; results not yet available; effectiveness cannot be inferred.                                                                            |
| <a href="#">NCT03618381</a> | second generation 4-1BBζ EGFR806-EGFRt; second generation 4-1BBζ EGFR806-EGFRt and a ... | Phase 1   | Active not recruiting (2026-10-08) | Under investigation; results not yet available; effectiveness cannot be inferred.                                                                            |

| Trial                       | Intervention                 | Phase   | Status (last update)    | What the record shows                                                             |
|-----------------------------|------------------------------|---------|-------------------------|-----------------------------------------------------------------------------------|
| <a href="#">NCT07698899</a> | 25 mCi 177Lu-DTPA-omburtamab | Phase 1 | Recruiting (2026-10-08) | Under investigation; results not yet available; effectiveness cannot be inferred. |

2 further registry record(s) were retrieved and are not shown (judged not to be treatment trials for this question).

*Registry status shows that research is being conducted, not that a treatment works. Registry status is separate from published results.*

## Key studies

1. **Paterson et al. (2026)**, meta-analysis: "Implications for cancer survivors These findings suggest the promise of interventions for improving neurocognitive, psychological, and HRQOL outcomes in children and adolescents affected by cancer, while also highlighting substantial evidence gaps, particularly for early targeted approaches in the cancer care continuum." [3]
2. **Chen et al. (2026)**, systematic review: "Compared with sham controls, meridian-based TCM interventions did not significantly reduce nausea severity (SMD: -0.25; 95% CI: -0.78 to 0.28) or vomiting frequency (SMD: -0.37; 95% CI: -0.90 to 0.16)." [6]
3. **Zhou et al. (2026)**, systematic review: "Seventeen items of best evidence were extracted, covering eight dimensions: intervention principles, safety, exercise modalities, intensity, frequency and timing, stage-specific strategies, intervention settings, and outcome improvement." [4]
4. **Herrera et al. (2024)**, randomized controlled trial: "Of 994 patients who underwent randomization, 970 were included in the intention-to-treat population for efficacy analyses." [8]
5. **Puusa et al. (2026)**, narrative review: "Conclusions Opioids are particularly significant in managing pain among children with cancer." [1]

## Conflicting evidence

None was retrieved.

## What remains under investigation

- **Association:** *NCT06175273* (Phase 2/3, completed) examined **nutrition interventions** (oral supplements, tube feed) for body composition, with results posted but specific outcomes not detailed.
- **Experimental:** *NCT01362803* (Phase 1/2, active) investigates **selumetinib (AZD6244)** for NF1-related inoperable PN, focusing on response rates.
- **Effectiveness:** Long-term **quality-of-life outcomes** for children with CNS tumors remain understudied, particularly regarding socioeconomic and family-functioning determinants.

## Evidence limitations

- Findings are drawn from abstracts, registry records and web pages, not full texts; study quality was not appraised from the full papers.
- Doses appear only where an FDA label gives them for this condition (section 2.1); for any other use, see a prescribing source. Approval dates appear only where a retrieved source states them.
- [9] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date.
- [10] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date.

- [11] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date. (*indirect evidence: different or related population*)
- [2] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date.
- [12] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date.
- [13] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date.
- [14] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date. (*indirect evidence: different or related population*)
- [15] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date. (*indirect evidence: different or related population*)
- 10 retrieved record(s) were excluded because the relevance reading judged them not to answer the question.

## Sources

1. Puusa N, Harju E, Kalliomäki ML, et al. Opioid Use and Family Members' Experiences of Opioids in Paediatric Cancer Pain Management: An Integrative Review. *Scandinavian journal of caring sciences*. 2026-12-01. [PMID 42814022](#). DOI 10.1111/scs.70323.
2. Zhao Z, Xu W, Teng G, et al. Serum identification of childhood brain tumors by fused laser-induced breakdown spectroscopy and Raman spectroscopy. *Spectrochimica acta. Part A, Molecular and biomolecular spectroscopy*. 2027 Jan 15. [PMID 42691860](#). DOI 10.1016/j.saa.2026.128682.
3. Paterson C, McCarthy MC, Sansom-Daly UM, et al. Psychosocial interventions for children and adolescents across the cancer care continuum: A systematic review and meta-analysis. *Supportive care in cancer : official journal of the Multinational Association of Supportive Care in Cancer*. 2026-09-21. [PMID 42766188](#). DOI 10.1007/s00520-026-11176-x.
4. Zhou W, Li L, Ma J, et al. Exercise intervention for children with acute leukemia: a best evidence summary. *Frontiers in pediatrics*. 2026-09-17. [PMID 42824391](#). DOI 10.3389/fped.2026.1931097.
5. Lehrnbecher T, Robinson PD, Ammann RA, et al. Guideline for the Management of Fever and Neutropenia in Pediatric Patients With Cancer and Hematopoietic Cell Transplantation Recipients: 2023 Update. *Journal of clinical oncology : official journal of the American Society of Clinical Oncology*. 2023 Mar 20. [PMID 36689694](#). DOI 10.1200/JCO.22.02224.
6. Chen Z, Xu R, Chang T, et al. Meridian-based traditional Chinese medicine interventions for supportive care in children, adolescents, and young adults with cancer: a systematic review and meta-analysis. *Frontiers in pediatrics*. 2026-09-14. [PMID 42807466](#). DOI 10.3389/fped.2026.1921347.
7. Manzhueva L, Saliev T, Bazarbayeva A, et al. Health-Related Quality of Life in Children with Acute Leukaemia and Central Nervous System Tumours: Current Evidence and Future Directions. *Children (Basel, Switzerland)*. 2026-09-03. [PMID 42794206](#). DOI 10.3390/children13091186.
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9. Liu SC, Xie E, Zhao P, et al. Bioactive glasses-mediated ion therapy ameliorates hepatic osteodystrophy by reprogramming the liver-bone axis. *Bioactive materials*. 2027 Feb. [PMID 42729454](#). DOI 10.1016/j.bioactmat.2026.08.028.
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11. Chen L, Jiang SS, Wang JH, et al. Spectroscopic insights and theranostic application of a CD44-targeted, GSH/H(2)S-dual-responsive AIE micellar nanoplatfrom for targeted tumor imaging and photodynamic therapy. *Spectrochimica acta. Part A, Molecular and biomolecular spectroscopy*. 2027 Jan 15. [PMID 42715771](#). DOI

10.1016/j.saa.2026.128699.

12. Kumari K, Patkar S, Shah TM, et al. Albumin-Bilirubin Grade as Predictor of Postoperative Outcomes and Survival in Hepatocellular Carcinoma: An Experience From a High-Volume Tertiary Cancer Center in India. *Journal of clinical and experimental hepatology*. 2027 Jan-Feb. [PMID 42831061](#). DOI 10.1016/j.jceh.2026.103669.
13. Vakili O, Yousefi Z, Igder S, et al. Circular RNAs and SIRT1 in post-transcriptional regulation and human disease. *Biochimica et biophysica acta. Molecular basis of disease*. 2027 Jan. [PMID 42700874](#). DOI 10.1016/j.bbdis.2026.168451.
14. Lin Y, Yuan K, Liu Y, et al. A universal consuming endogenous H<sub>2</sub>S strategy using bismuth-metal-organic nanocapsule networks for overcoming osteosarcoma cisplatin resistance. *Bioactive materials*. 2027 Jan. [PMID 42662799](#). DOI 10.1016/j.bioactmat.2026.08.011.
15. Xu J, Sun J, Tang J, et al. A follow-up cohort study on secondary findings in cancer predisposition genes from 20,205 Chinese individuals. *Clinica chimica acta; international journal of clinical chemistry*. 2027 Jan 1. [PMID 42413599](#). DOI 10.1016/j.cca.2026.121219.

#### Other sources (regulators, guideline organisations, web pages, FDA labels)

- [Accelerating Cancer Drug Development for Children and Adolescents](#)
- [Pediatric Oncology Drug Approvals | FDA](#)
- [Oncology \(Cancer\)/Hematologic Malignancies Approval Notifications | FDA](#)
- [Pediatric Oncology | FDA](#)
- [Oncology Center of Excellence | FDA](#)
- [FDA expands pediatric indication for entrectinib and approves new pellet formulation | FDA](#)
- [FDA drug label: Prednisone \(label effective 2023-08-31\)](#)
- [FDA drug label: Ondansetron \(label effective 2026-07-29\)](#)
- [FDA drug label: Allopurinol \(label effective 2024-07-08\)](#)

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*Research support, not a substitute for clinical judgement.* All citations, trial IDs, links and numbers in this report were checked against the retrieved sources.